

LIVER GENOMICS · SINGLE-NUCLEUS RE-ANALYSIS

No transcriptional de-zonation signal in matched MASLD biopsies

— *but the pericentral detox program dims*

A critical re-analysis of the single-nucleus RNA-seq in Gribben et al., Nature 2024 (GSE202379)

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Computational Genomics (76553) · Hebrew University of Jerusalem · Hackathon

47 donors · ~99,809 nuclei · 69,426 hepatocytes · raw-count, donor-level analysis

Hepatocytes are transcriptionally zoned

Pericentral (zone 3)

Periportal (zone 1)

Central vein

Portal tract

GLUL, CYP3A4, CYP2E1 — drug metabolism (Wnt-driven)

ASS1, CPS1, PCK1, HAL, ALDOB — urea cycle

THE PUBLISHED CLAIM — Gribben 2024

“Hepatocytes lose their zonation.”

- End-stage hepatocytes co-express pericentral + periportal markers — validated by imaging + protein.
- Hepatocyte↔cholangiocyte transdifferentiation, “prominent in end-stage.”

OUR QUESTION

Does hepatocyte zonation degrade across MASLD?

— and is it linked to the hepatocyte → cholangiocyte transdifferentiation they describe?

The catch

our first approach is misleading — and the tissue itself is confounded

Catch 1: A relative ruler — and what it hides

THE OLD RULER

We first read it with a **z-scored zonation coordinate** and **marker-marker correlations**, pooled across all samples.

WHAT IT SHOWED

“Hepatocytes lose their zonation; the pericentral program turns off, strongest at end-stage.”

THE TRAP

Hides who moved

no counts — which cells? how many? and from where?

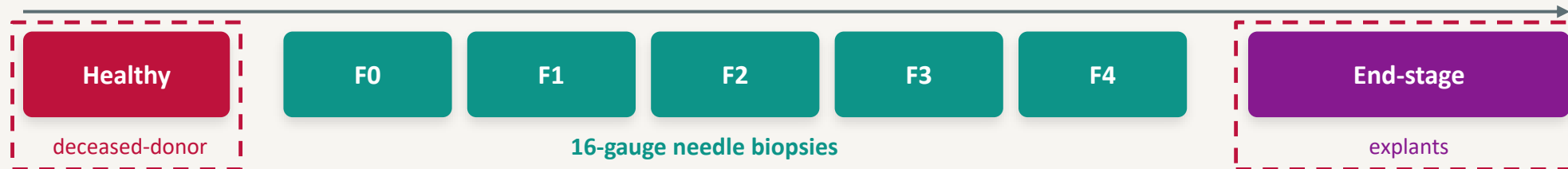
Bent by artifacts

depth, cell number and tissue source all tilt a z-score or correlation.

→ so we count molecules instead.
Pivoting back to the paper’s raw data.

Catch 2: Stage is entangled with tissue procurement

① The two extremities are rouge - acquisition isn't needle biopsy



The ends aren't acquisition-matched to the biopsies (2/8/12/12/4) → we use biopsy-only F1–F4.

② And they also carry organ-wide stress

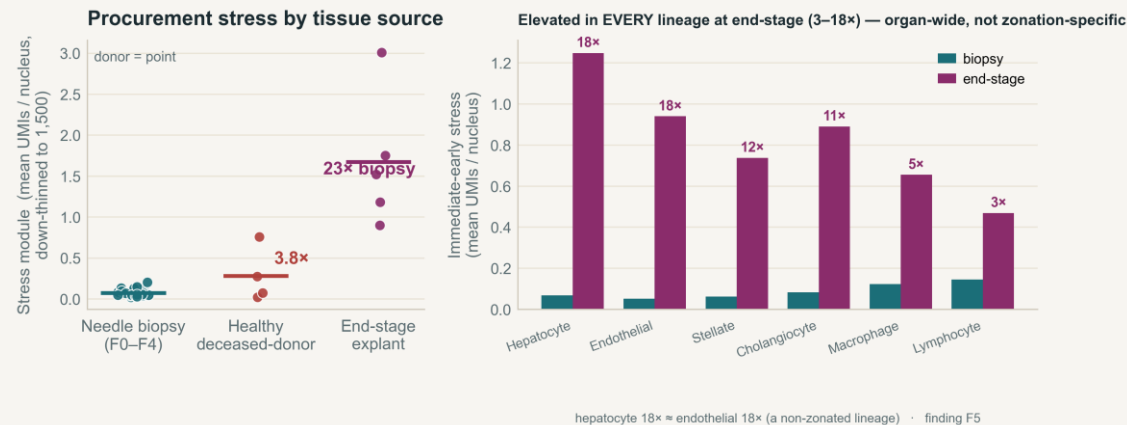


Figure 1. Procurement-stress module by tissue source (donor-level).

Stress spikes even where there's no zonation.

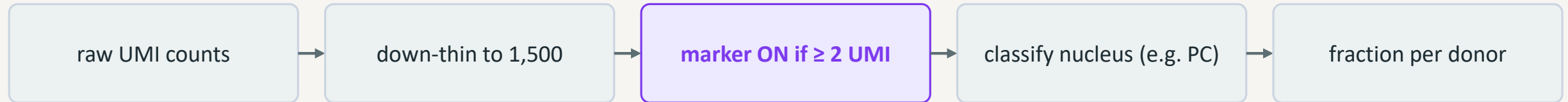
Immediate-early / heat-shock genes jump 18× in the ends — but **endothelium, which has no zonation program, jumps 18× too**; hypoxia (HIF) stays flat. So it's not necessary zonation change affect — but rather, at least as probable - acute handling¹.

¹ Dissociation / handling artifact: van den Brink et al. 2017; O'Flanagan et al. 2019; Denisenko et al. 2020. Provenance: paper Methods + GSE202379 metadata.

Method – corrected

how we re-test it, in counts

Raw-count anchor classification



Raw counts, not Single Cell Transformation (SCT)

A single molecule may be ambient — absolute detection is what matters.

Depth-matched¹

Down-thin every nucleus to 1,500 UMIs so depth can't drive detection (stable at 1k / 1.5k / 3k).

≥ 2 UMI, not ≥ 1

Kills ambient soup — apparent co-expression 7–10% $\rightarrow$ 0.2–0.5%.

Donor is the unit (~47)

No pseudoreplication. Robust on 6 marker sets (2–1,637 genes, incl. Paper 2's own).

¹ Depth-matched down-sampling: Amezquita et al., Nat. Methods 2020. · Raw-extraction integrity: 9/9 sanity checks pass (integer UMIs; 69,426 hepatocytes match the paper).

Four classes – and how they can move

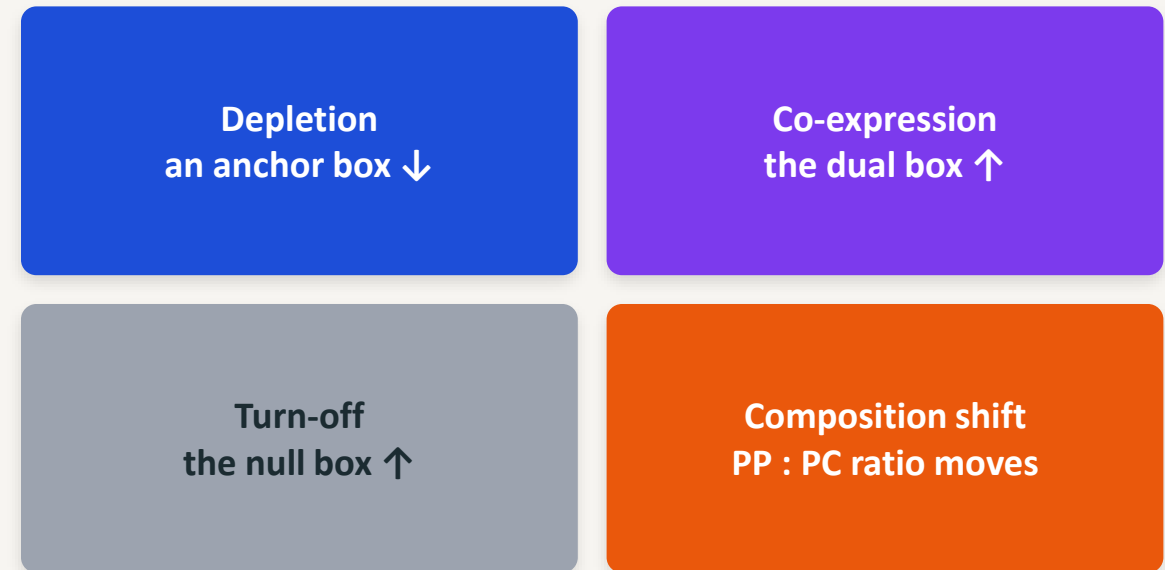
Every hepatocyte is one of four classes (left).

“Losing zonation” = cells redistribute across them — four distinct moves (right), each with its own count signature.

THE FOUR CLASSES



THE FOUR MOVES



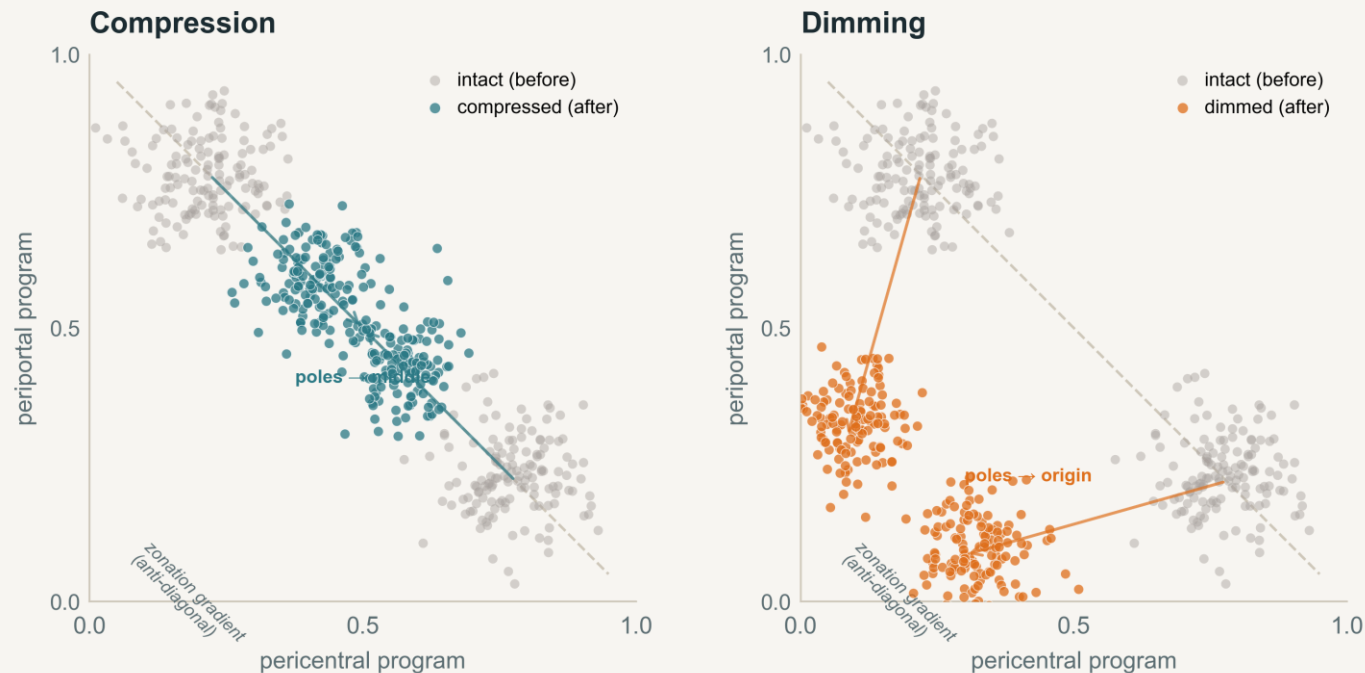
The moves are dissociable — an emptying anchor box (depletion) and a filling null box (turn-off) are opposite directions; neither implies the other.

A single z-score or correlation collapses all four into one number — only counting molecules tells them apart.

Beyond the classes — zonation is a gradient

Zonation is a continuous gradient — cells strung on an anti-diagonal. It can weaken two ways **without any cell changing class**:

Two de-zonation modes in anchor-program space · SCHEMATIC (illustrative, synthetic cells)



Compression — *shape*

Balance drifts to the middle; the poles' contrast shrinks.

Dimming — *level*

The pericentral detox program declines in level — same balance, lower volume.

The analysis stack, end to end

Every quantitative claim runs at the donor level on raw molecule counts. The methods, grouped by step:

COUNTS & DEPTH

- **Raw UMI extraction (not SCT)** — absolute detection needs molecules, not normalized residuals — Seurat / Matrix (R)
- **Binomial down-sampling → 1,500 UMIs / nucleus** — removes sequencing depth as a confound (the depth-match)

PER-GENE DGE

- **Pseudobulk, donor-level** — sum each donor's hepatocyte counts → one profile per donor (Squair 2021)
- **edgeR — TMM + NB quasi-likelihood GLM** — composition-robust normalization + per-gene fibrosis trend (filterByExpr gene filter)

GENE-SET TESTS

- **GSEA pre-rank — gseapy** — is a set enriched at one end of the ranked genome?
- **CAMERA — competitive (limma)** — set shifted more than the rest of the transcriptome? (Wu & Smyth 2012)
- **ROAST / mroast — self-contained (limma)** — does the set move against zero? rotation p-value (Wu 2010)

SOURCE & STATISTICS

- **decontX — ambient-RNA removal** — models native + ambient mixture, subtracts the estimate (celda; Yang 2020)
- **TOST equivalence · Cramér's V · Spearman / partial corr.** — bound the null · test batch confounding · trend & covariate control

Stack: R (edgeR, limma, decontX/celda, Seurat) · Python (numpy, scipy, pandas, gseapy). Refs: edgeR / TMM — Robinson, McCarthy & Smyth 2010; Robinson & Oshlack 2010 · CAMERA — Wu & Smyth 2012 · ROAST — Wu et al. 2010 · decontX — Yang et al. 2020 · pseudobulk — Squair et al. 2021 · GSEA — Subramanian et al. 2005.

Results

what the counts actually say – and a meaningful differential gene expression analysis

No depletion signal across matched biopsies

Matched biopsy fibrosis F1–F4 · each point = one donor; bar = stage mean $\pm$ SEM (F0 n=2 omitted; full F0–F4 in backup)

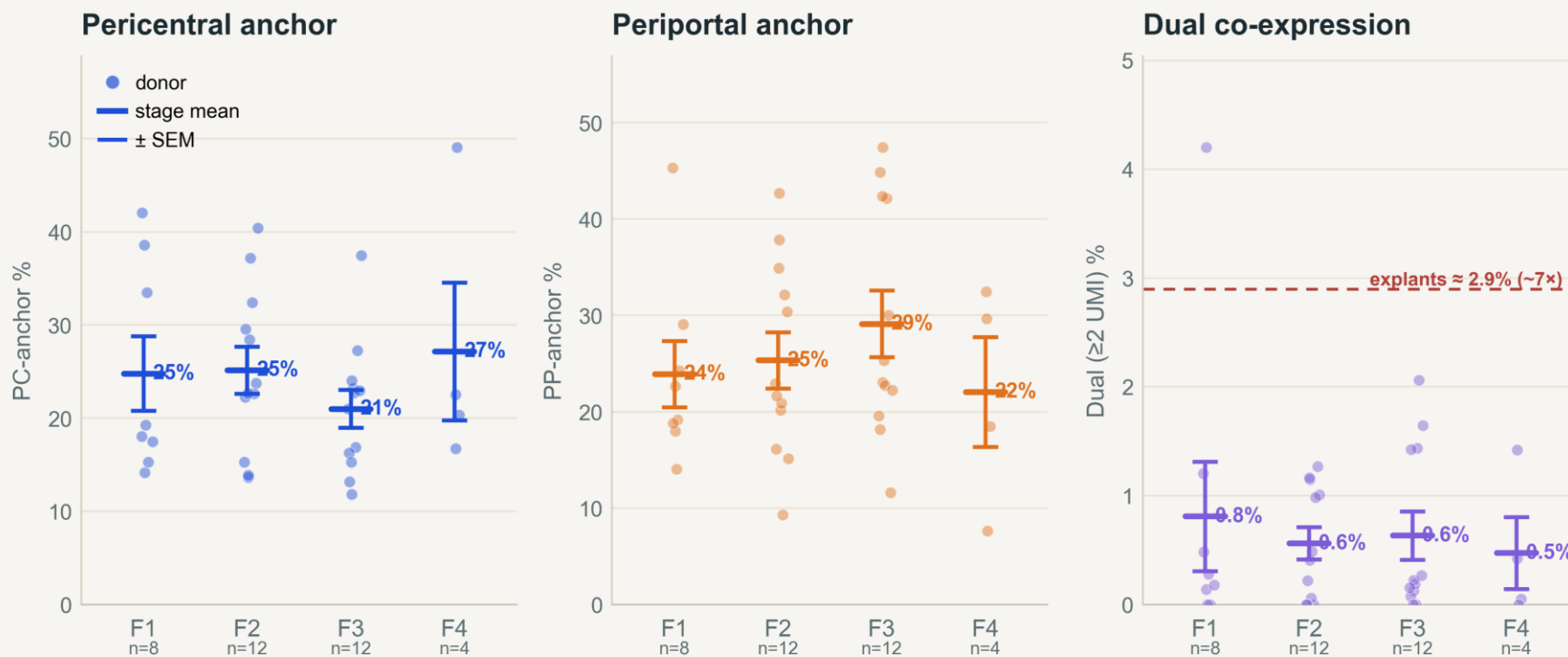


Figure 2. Pericentral / periportal anchor and dual co-expression across matched biopsy F1–F4 (each point = one donor; bar = stage mean $\pm$ SEM; F0 n=2 omitted — in backup).

No depletion.

PC & PP anchors flat across F1–F4.

No co-expression.

Dual stays $\sim 0.4\%$ per cell (vs 2.9% in explants).

Null & PP:PC flat too.

40/40/40/41%; ratio ≈ 1.1 – 1.7 .

→ no transcriptional de-zonation signal.

Equivalence (TOST): a $> \pm 19$ pp F4–F1 shift is excluded; a ≤ 10 pp drift is not (F4 n=4). Spread + 10-confounder audit in backup.

Cells stay spread across the zones — no apparent collapse

Each panel: where hepatocytes fall on the pericentral ↔ periportal axis. $PC / (PC + PP)$ directly from counts. A zonal gradient histogram. A spread of positions = zonation present; cells bunched into one pile = collapse.

Per-cell zonal balance by stage — the distribution barely moves across biopsy F1–F4 (dashed = F1 reference); only the confounded explant collapses to one pole

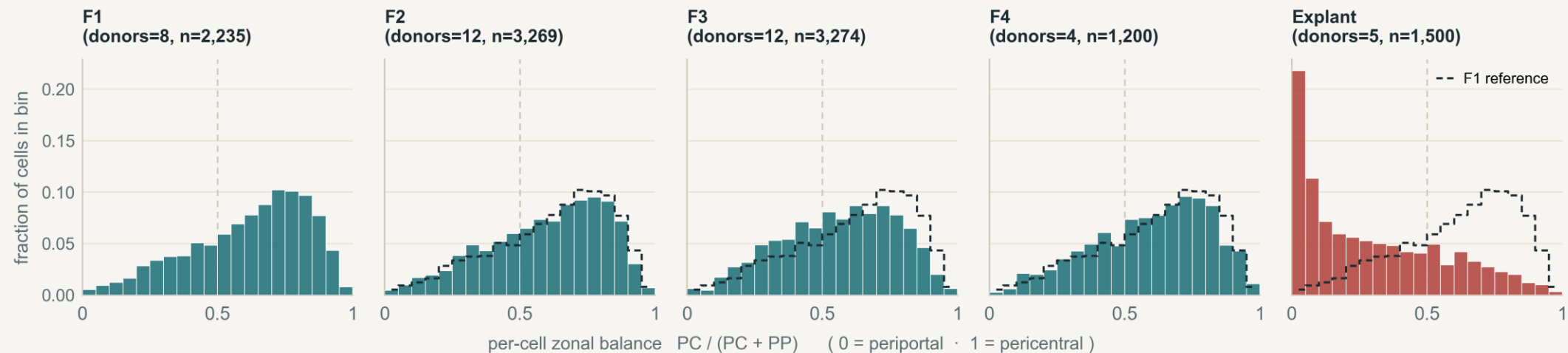
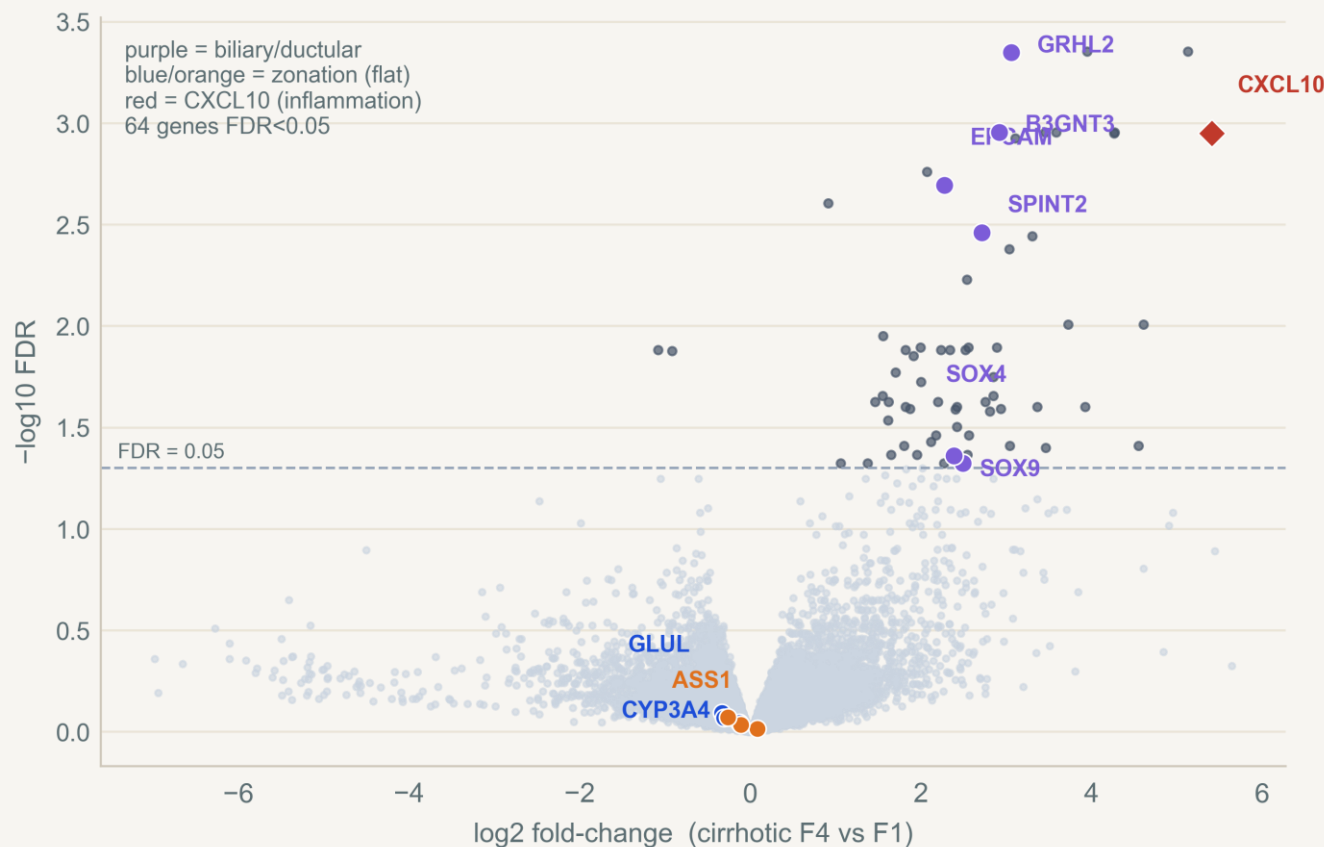


Figure 3. Per-cell zonal balance by stage (down-thinned to 1,500 UMIs, ≤ 300 nuclei/donor).

Dashed outline = the F1 distribution, repeated on each panel: biopsy F1–F4 barely move; only the explant collapses to the periportal pole.

Across biopsy F1–F4 the mass stays spread — the share of cells landing across [0.4–0.6) drifts only 22 → 24 → 28 → 26% (peak F3, then reverts), non-monotone and mild. **Only the confounded explant collapses to a single (periportal) pole.**

Per gene, nothing usable moves



64 of ~21,000 genes pass FDR¹

— mostly biliary / ductular.

Zonation genes flat (GLUL FDR 0.80).

No large single-gene program.

But a weak program, spread thinly across many genes, hides from per-gene FDR

Figure 4. Genome-wide pseudobulk differential expression, cirrhotic F4 vs F1 (edgeR).

¹ Pseudobulk: Squair et al. 2021; edgeR / TMM: Robinson, McCarthy & Smyth 2010; Robinson & Oshlack 2010.

The biliary signal: compositional? a lead

What we see: biliary / ductular genes (EPCAM, SOX9, GRHL2, B3GNT3, SPINT2) rise at F4 — exactly where cirrhosis’s ductular reaction floods the tissue with cholangiocytes.

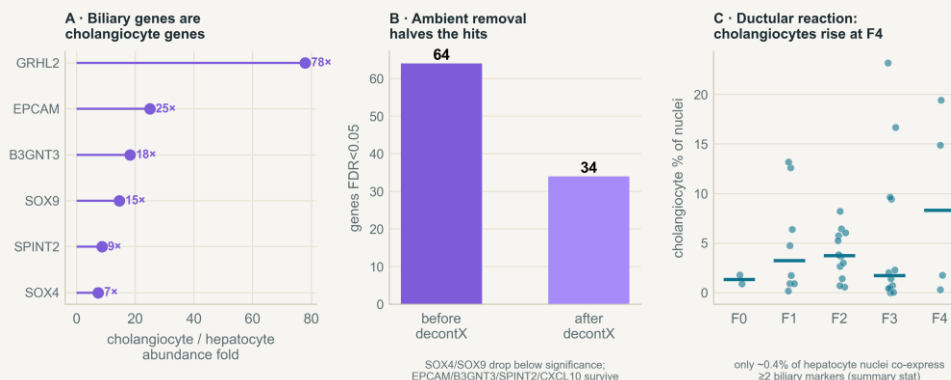


Figure. Source attribution — cross-lineage abundance, decontX ambient-RNA removal, and the F4 ductular reaction.

COMPOSITIONAL (the lead)

These are cholangiocyte genes. At F4 there are more cholangiocytes → more of their RNA in the ambient “soup” (plus the odd doublet) enters hepatocyte droplets → hepatocytes look biliary without transdifferentiating.

RARE PER CELL (read carefully)

Only ~0.4% of hepatocyte nuclei co-detect biliary markers at ≥2 UMI — not population-wide. But that is prevalence over ALL hepatocytes: it argues against MASS transdifferentiation, not against a rare sub-population (class imbalance).

OPEN (a residual survives)

decontX removes about half the hits (64 → 34), but EPCAM / SPINT2 / B3GNT3 survive decontamination — ambient can’t explain all of it, so a rare intrinsic state is not excluded.

Verdict: most consistent with composition / ambient (the ductular reaction) — a lead, not a closed verdict.

Pericentral detox dims — identity stays

Cells keep their class — but within pericentral nuclei, the detox program's level falls with fibrosis

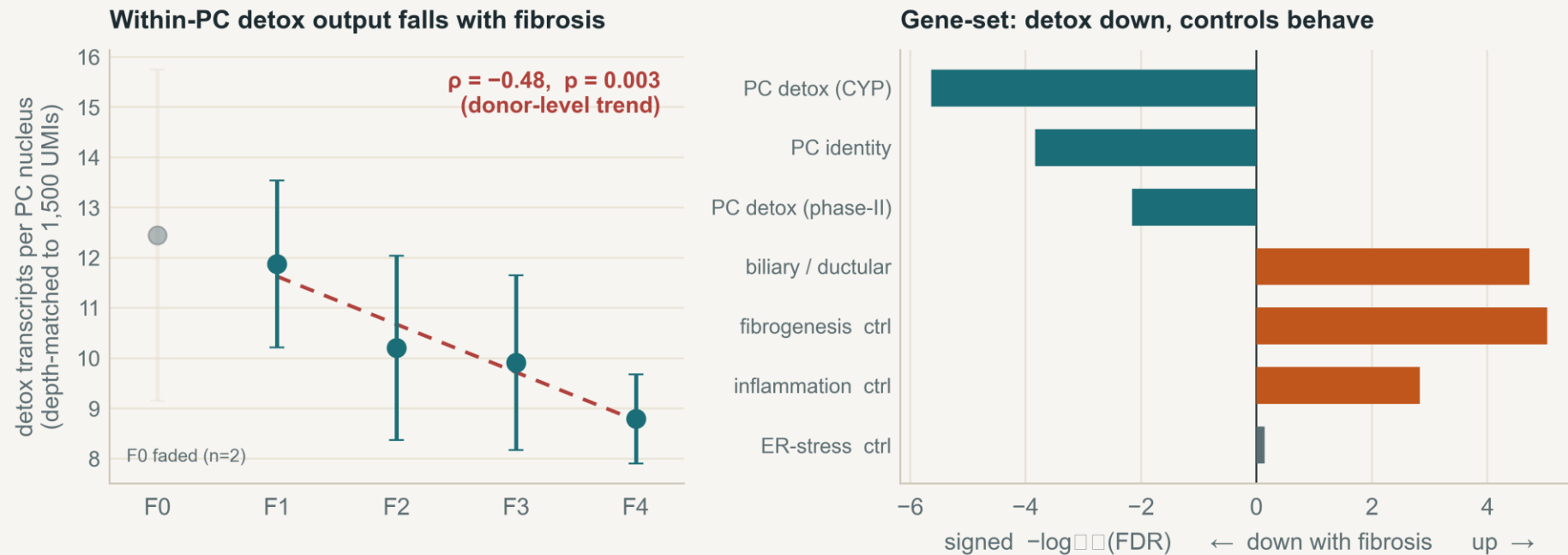


Figure 5. Left — within-PC detox output¹ (depth-matched transcripts per PC nucleus) across fibrosis. Right — gene-set programs²: PC detox / identity down, biliary + fibrogenesis up, validating controls behaved.

Like a radio: the genre held (cells keep identity); the volume dropped (the program dims). A relative decline (transcripts within a fixed budget), not proven absolute molecule loss; a donor-level trend; could partly reflect a shift among PC subzones. No single gene clears per-gene FDR — only the coordinated set + the per-cell trend.

¹ Within-PC detox output: depth-matched to 1,500 UMIs, donor-level Spearman ($\rho = -0.48, p = 0.003$); measured on detox genes (CYP2E1, CYP1A2, ADH4...) disjoint from the GLUL/CYP3A4 identity anchors — so the anchor fraction stays flat while the module dims. ² Gene-set: camera — Wu & Smyth 2012; ROAST — Wu et al. 2010; GSEA — gseapy.

Zonal identity holds — the pericentral detox program dims

01 Cells keep their zonal genomic identity — no de-zonation signal

02 The one real change: the pericentral detox program dims

03 The dramatic trajectory **possibly** tracks acquisition, not disease

We correct one evidence leg —

the snRNA marker-correlation; the paper's imaging and protein staining are untouched.

The biliary signal is a separate compositional lead.

Are you overturning the paper? No

- We re-tested only the snRNA-seq transcriptional zonation/plasticity evidence.
- We did NOT test imaging, protein staining, organoids, or 3D architecture.
- Claim: matched biopsy transcriptomes do not show progressive zonation loss.
- We agree the strong signal is an end-stage phenomenon.

Why a flat result is readable despite huge donor spread

Within each fibrosis stage the per-donor pericentral-anchor fraction spans ~25–35 pp (SD $\approx$ 12 pp).

Could that scatter be hiding a real trend? No — four reasons:

1 The donor is the replicate — not the cell

Inference is donor-level ($F1-F4 = 8 / 12 / 12 / 4$). A stage MEAN has standard error $SE \approx SD/\sqrt{n} \approx 12/\sqrt{10} \approx 4$ pp — we compare stage means (± 4 pp), not individual donors (± 12 pp).

2 The spread is the same at every stage

Variance is homoscedastic — as wide at F1 as at F4. A real progression would shift the mean monotonically beyond ± 4 pp; it doesn't. This is noise around a flat line, not a trend buried in noise.

3 We bound what is excluded (TOST)

Not merely “no trend found”: a shift $> \pm 19$ pp between F1 and F4 is positively rejected at the donor level. Only a subtle drift (≤ 10 pp) remains possible — limited by F4's $n = 4$.

4 Stable across marker sets & thresholds

The flat result holds on 6 marker sets (2–1,637 genes, incl. Paper 2's own) and the full $\geq 1 / \geq 2 / \geq 3$ -UMI grid — no setting-dependent signal hiding in the variance.

Bottom line: large person-to-person variance $\neq$ an undetectable signal. With donor-level inference and an equivalence bound, a large progressive de-zonation is **excluded** (a subtle ≤ 10 -pp drift is not).

Null is flat; the PP : PC ratio shows no stage trend

Secondary endpoints across biopsy F1–F4 · each point = one donor; bar = stage mean ± SEM

Null (double-negative): $\approx 40 / 40 / 40 / 41\%$
 — no rise across stages.

PP : PC ratio: $\approx 1.20 / 1.25 / 1.67 / 1.07$ —
 near the 1 : 1 line (F3 pulled by one high donor).

Same large person-to-person spread as the anchors, same absence of a stage trend.

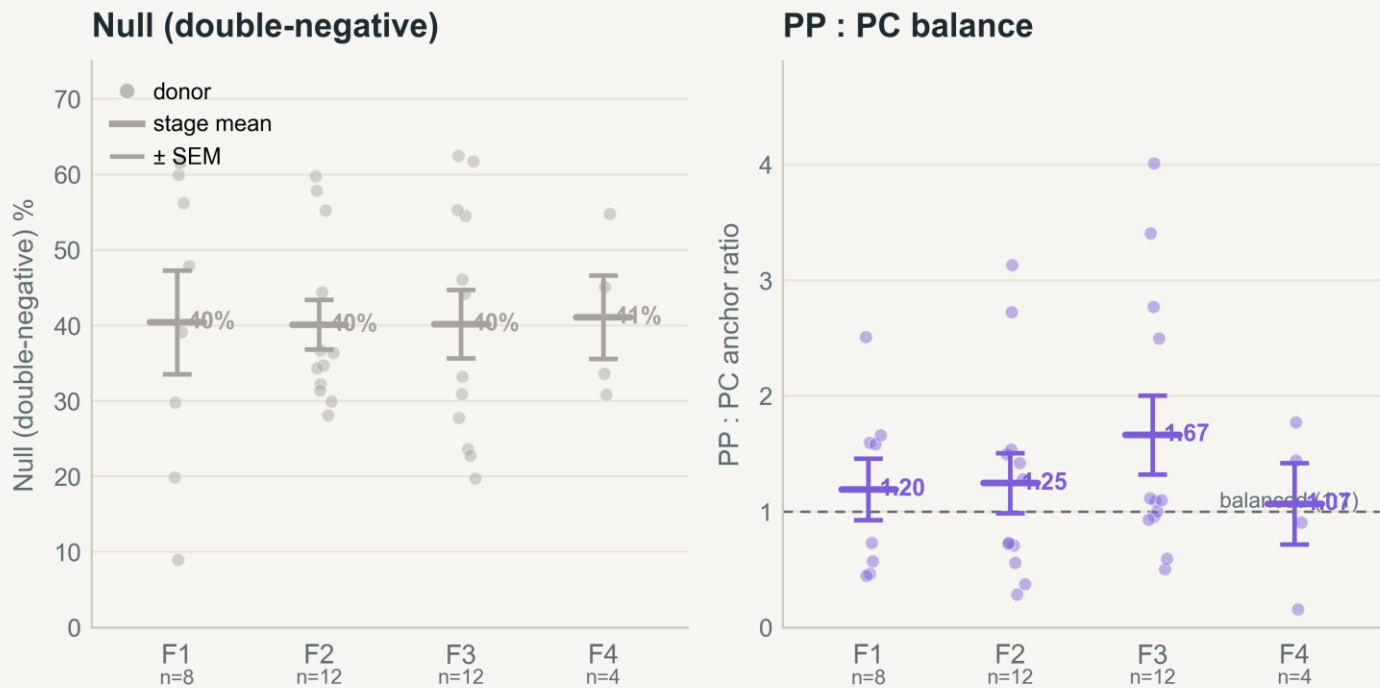
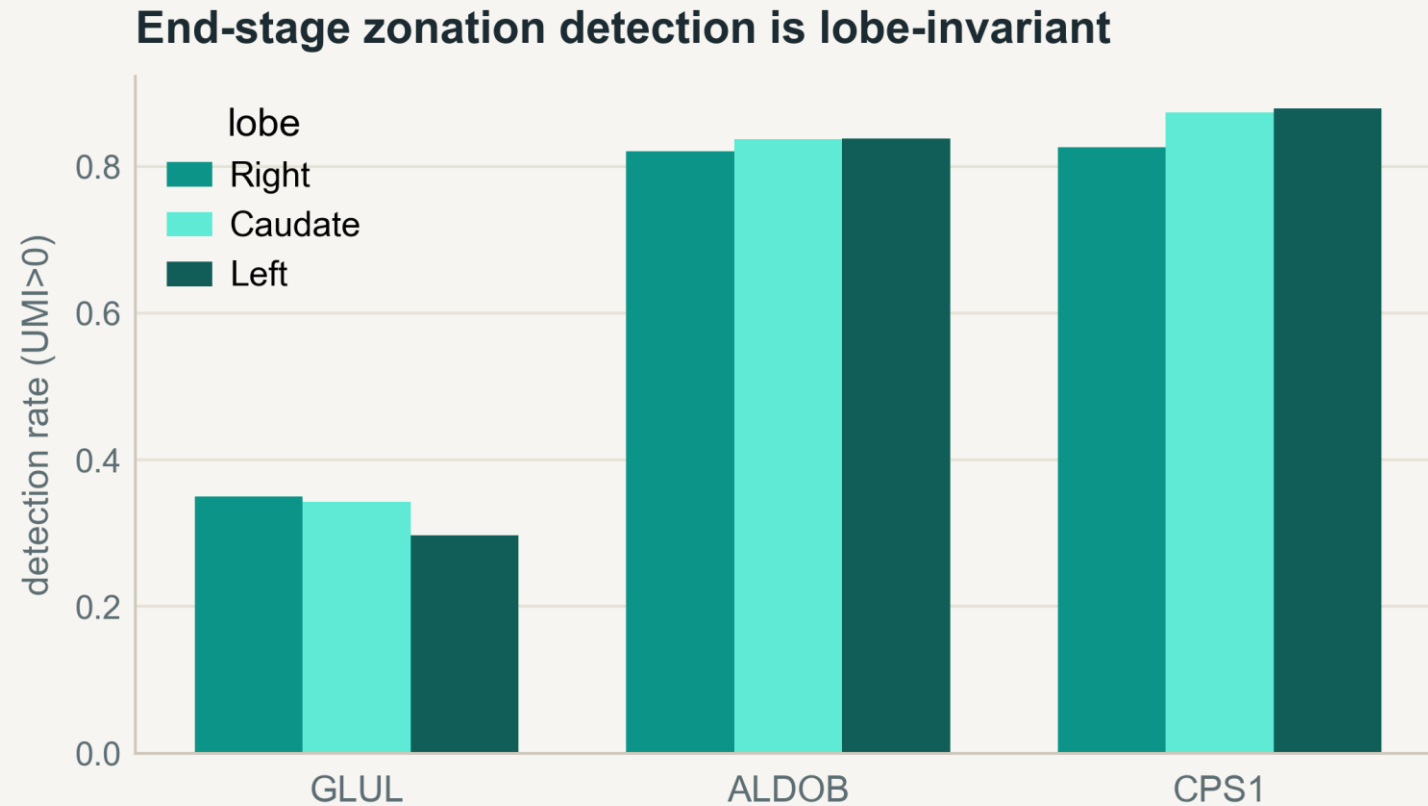


Figure 7. Null (double-negative) fraction and the periportal : pericentral anchor ratio across biopsy F1–F4 (each point = one donor; bar = stage mean ± SEM).

Right, caudate and left lobes all agree

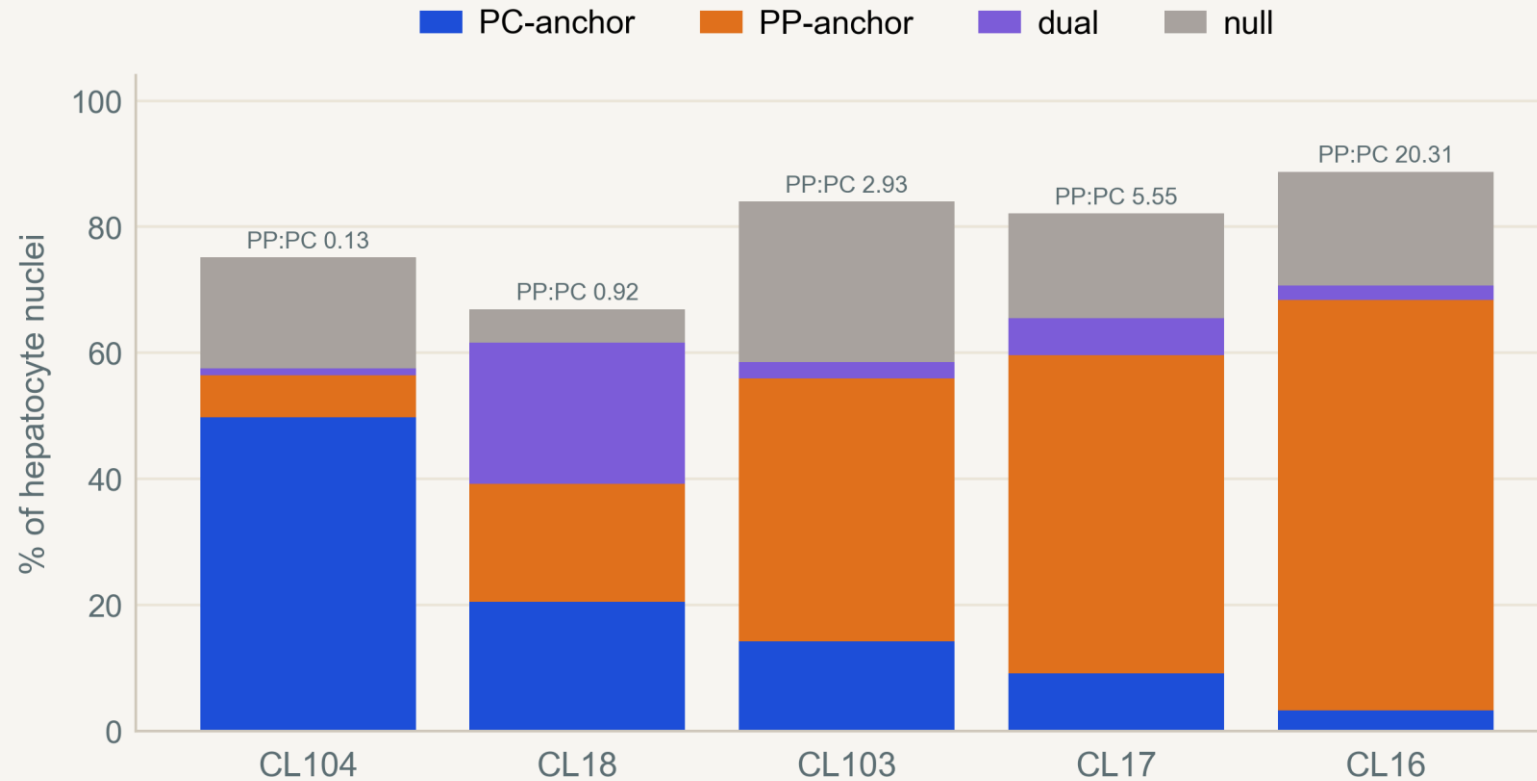


Detection matches across all three lobes.

Lobe does not explain the signal; explant-vs-biopsy acquisition still does.

Figure 8. Zonation-marker detection across Right / Caudate / Left lobes in end-stage explants.

End-Stage Shenanigans: Five donors, five phenotypes



PC-anchor 3% → 50%; PP:PC 0.13 → 20.

Dramatic but NOT one program — pooling manufactures a uniform “collapse.”

Figure 9. Per-explant anchor fractions — end-stage is heterogeneous, not one coherent program.

Sensitivity grid + equivalence bound

Stable across every variant:

anchor genes, periportal rule, ALDOB in/out, CPS1-based, 1 vs 2 UMI.

No depletion · no dual rise · no turn-off — in all.

Bound (TOST): large shift (~19 pp) excluded; subtle drift ≤ 10 pp not (F4 n=4).

We exclude a large shift, not subtle drift

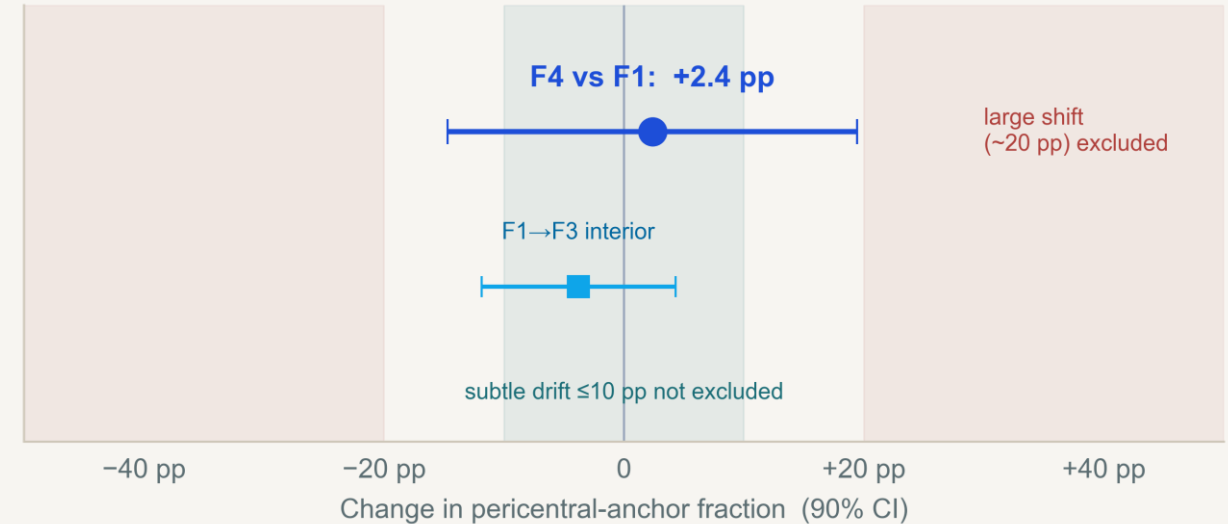


Figure 10. Equivalence bound on the F4-vs-F1 pericentral-anchor change.

Confounder battery — the numbers

Confounder	What we did (test · numbers)	Verdict
Tissue source	organ-cube / explant ends are not acquisition-matched to needle biopsy	excluded → biopsy-only F1–F4
Procurement stress	IEG+HSP across 6 lineages; hep 18× ≈ endothelial 18× (no zonation)	organ-wide handling, not zonation
Sequencing batch	run × source Cramér’s V = 0.84; run × F (biopsy) = 0.40	ends confounded; biopsy estimable
Lobe (R / C / L)	marker detection across right / caudate / left lobes in explants	lobe-invariant
Sequencing depth	binomial down-thin to B = 1,000 / 1,500 / 3,000 UMIs	PC-share flat at every B (SD 0.006–0.010)
Ambient RNA (“soup”)	corr(ALB share , dual fraction) across 38 donors = +0.04	does not drive co-expression
Cholangiocyte mis-annotation	KRT19+ ≤ 0.001, EPCAM+ ≤ 0.003 in every anchor class	not contaminating cholangiocytes
Ploidy / complexity	nFeatures by stage: 2218 / 3063 / 3169 / 2641 / 1984	non-monotone — does not track stage
Depth-match discard	3.5–7.8 % dropped below B, uniformly lower-detection	depth-driven, not PC-biased
Clinical covariates	partial corr(detox , F Age) = −0.32; Age–F = +0.43	demographics do not explain trends

Why raw counts, why pseudobulk

- Raw counts: marker co-detection + ambient sensitivity need molecules, not SCT residuals.
- Pseudobulk: cells from one donor are not independent — sum per donor, then edgeR (Squair 2021).
- edgeR: filterByExpr → 21,022 genes; TMM; NB quasi-likelihood GLM, robust dispersion; common BCV 0.405.
- Primary signal persists in DIRECTION with a run covariate and within a single run; significance attenuates with n (not a batch artifact).

Does decontX prove contamination? No

- decontX models a native + ambient RNA mixture and subtracts the estimate.
- Hits 64 → 34; SOX4/SOX9 drop below significance → supports an ambient contribution.
- But it can remove true hybrid signal OR leave residual contamination.
- Source attribution is a lead, not a closed verdict; CXCL10 is a separate candidate inflammatory signal.

Ten pre-specified sets — the detox program goes down

Gene set	Zone / program (identity)	n	Dir	camera FDR
xenobiotic_CYP	PC detox — phase-I (CYP)	13	↓	2×10 ⁻⁶
detox_phase2	PC detox — phase-II (UGT/GST/SULT)	16	↓	7×10 ⁻³
pericentral_anchors	PC identity (Paper 2 landmarks)	20	↓	1.5×10 ⁻⁴
periportal_anchors	PP identity (Paper 2 landmarks)	20	↑	1.5×10 ⁻²
urea_cycle	PP program (nitrogen)	9	↓	0.12 (ns)
bile_acid_lipid	mixed metabolism	20	–	0.72 (ns)
cholangiocyte_ductular	biliary / bile-duct markers	10	↑	1.9×10 ⁻⁵
CTRL_interferon	inflammation (+control)	20	↑	1.5×10 ⁻³
CTRL_EMT	fibrogenesis / scar (+control)	20	↑	9×10 ⁻⁶
CTRL_ER_stress	unfolded-protein resp. (–control)	20	–	0.72 (ns)

Donor-level, biopsy F0–F4, limma::camera (GSEA agrees). Pre-specified, no fishing: PC/PP identity = Paper 2’s own landmark genes; CYP / detox / urea / bile = KEGG·HGNC cores; biliary = plasticity.txt + markers; controls = MSigDB Hallmark cores. The validating controls behaved (fibrogenesis & inflammation ↑, ER-stress flat) — proof the test detects real biology, not noise. The pericentral detox + identity programs fall; the see-saw (PP ↑) and biliary ↑ are why we adversarially controlled for composition (it survived).

Gene-set testing, in one breath each

Pseudobulk — sum each donor's hepatocyte counts into one profile per donor (donor = unit of inference).

CPM — each gene as a share of the donor's total counts; simple, but compositional (a few surging genes shrink everyone else's share).

TMM — the scaling factor is taken from the bulk of genes after trimming the extreme movers, so surging genes can't set it → composition-robust.

Per-gene DGE — test each gene on its own for a fibrosis trend (edgeR); misses a weak shift shared by many genes.

Gene-set test — test a whole pre-specified set at once. camera = competitive (more shifted than the rest of the genome?; Wu & Smyth 2012); ROAST = self-contained (does the set move against zero?; Wu et al. 2010).

Within-PC detox output — detox transcripts per pericentral nucleus after equalizing read depth: per-cell, relative to the cell's own output, immune to between-cell composition.

Every de-zonation route, mapped to its signature

Mechanism	Signature	Biopsy F0→F4 (donor-median)	Verdict
Pericentral depletion	PC-anchor fraction ↓	36 / 19 / 23 / 22 / 21 %	flat → no depletion
Periportal depletion	PP-anchor fraction ↓	20 / 21 / 22 / 24 / 24 %	flat → no depletion
Dimming	within-PC level ↓ (magnitude)	11.9 → 8.8 detox UMI/nuc (F1→F4); $p = -0.48$	REAL — gene-set + within-PC confirm (CYP FDR 2e-6)
Co-expression	dual (≥ 2 UMI) fraction ↑	~0.0 / 0.2 / 0.4 / 0.2 / 0.2 %	ambient soup → no
Gradient compression	per-cell balance → middle	mild, non-monotone (peak F3)	gradient present
Turn-off	null (double-neg) fraction ↑	34 / 44 / 36 / 39 / 39 %	flat → no turn-off
Composition shift	PP : PC anchor ratio	0.62 / 1.16 / 1.01 / 1.10 / 1.18	~flat → no shift
Induction	program level ↑	flat in biopsy; rises only end-stage	explant-only

Each row is a distinct way zonation could fail, mapped to its signature on depth-normalized counts. The structure-redistribution routes (cells crossing boxes) are all flat — cells keep their identity. The one magnitude route, dimming, is REAL: the pericentral detox program quiets, confirmed by gene-set + within-PC testing.

Conclusions • limits • next steps

Conclusions

- Full healthy→end-stage trajectory is source-confounded.
- In biopsy F1–F4 cells keep zonal identity — no de-zonation (large shift excluded).
- Two real biopsy-internal signals: a compositional biliary signal (a lead), and a confirmed pericentral-detox dimming (gene-set + per-cell).

Limits

- “PC-like program,” not lobular location — counts can’t see spatial position.
- Imaging / protein / organoid evidence not re-analyzed.
- F4 has only 4 biopsy donors.
- Detox dimming is a RELATIVE decline (not proven absolute) and could partly reflect a shift among PC subzones; self-contained test clears only the CYP core.

Next

- Spatial / independent-biopsy validation of the detox-dimming gradient.
- Leave-one-F4-donor-out DGE.
- Quantitative contamination model for the biliary lead.
- Functional readout of pericentral detox capacity.